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June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare and Medicaid Services

Re: Request for Information on Effective and Responsible Adoption of Technology, CMS-0042-NC.

Dear Administrator Oz,
Thank you for the opportunity to provide input on how effective and responsible adoption of technology can empower patients to make better decisions for their health and well-being. Please find our views on how expanding access, specifically to ***digital mental health tools (DMHTs)***, offers ways to support individuals in managing their mental health; supports clinicians in detecting, monitoring, and treating these conditions; fills existing gaps in the behavioral health workforce; and aligns with the Administration's comprehensive strategy to make America healthy again.

In this memo, we have also included recommendations on actions the Administration could take with existing authorities to improve patient outcomes, reduce regulatory confusion, and increase the efficiency of the multi-agency response to the mental health crisis. These recommendations stem from a multi-stakeholder process facilitated by Leavitt Partners. You can find more information on the project and the full report here.

Mental health and substance use disorders (MH/SUD) continue to create a drain on U.S. population well-being and productivity. Addressing unmet mental health and substance use disorder needs must be central to the Department of Health and Human Services' (HHS) comprehensive strategy to make America healthy again.

DMHT is at a pivotal point in its regulatory positioning in the U.S. and has the potential to be an effective tool to advance access to MH/SUD care. Consumers and health care professionals need a way to assess the multitude of products on the market and determine what works and what doesn't. Further, the U.S. regulatory environment risks falling behind if it does not keep pace with the development of technology such as DMHT.

There is a critical opportunity to boost access to evidence-based DMHT by creating a more modern and efficient regulatory framework that provides both greater clarity and guidance to vendors, as well as increased transparency and awareness for consumers and purchasers. The federal government should work in collaboration with private sector organizations to address barriers and ensure access to evidence-based DMHT as a key component in improving outcomes, well-being, and productivity in our nation.

Impact of Mental Health and Substance Use Disorders on Overall Health and the Economy

In recent data from Mental Health America, nearly one in four adults—roughly 60 million Americans—reported a mental illness in the past year, with the highest rates in completely rural counties.[1] In the same time frame, one in five adolescents had a major depressive episode. Many facing these challenges

are not able to get the help they need due to costs of care, lack of coverage, or lack of access to behavioral health professionals. These unaddressed mental health concerns reduce worker productivity. In a 2024 poll of full-time workers, 35 percent reported that mental health interferes with their ability to work.[2] This is consistent with research studies linking poor mental health with lost productivity (i.e., absenteeism and presenteeism)[3] and other data showing that poor mental health contributes to increases in leaves of absence, accommodations, and legal battles.[4] Conversely, mental health care can help reduce overall costs of care. For people with new behavioral health diagnoses, outpatient treatment reduced overall medical and pharmacy costs.[5] Among adults with physical health conditions, receipt of mental health care was associated with lower overall costs of care.[6]

Digital Mental Health Technology

A wide variety of digital mental health technologies (DMHTs) help individuals manage their mental health issues and support clinicians in detecting, monitoring, and treating these conditions. Examples range from digital therapeutics that are intended to treat a specific mental health condition to other patient-facing tools that can be used in screening, diagnosis, monitoring, and support. DMHT also includes tools intended to support clinicians in delivering clinical care as well as tools for health systems and others in managing mental health care. These tools are an important ingredient in enhancing, supplementing, or supporting care,[7] especially given the critical shortage of trained behavioral health providers in this country.[8]

In 2020, it is estimated that more than 90,000 new digital health apps were released. The majority focused on "wellness management," which means they are not subject to FDA oversight.[9] Despite the proliferation of digital tools, the lack of clear guidance on the safety, efficacy, and privacy of these tools has thwarted their wide adoption. Purchasers, including the Centers for Medicare and Medicaid Services (CMS), are looking for clear data on impact and outcomes to inform coverage and payment decisions. From the perspective of DMHT developers, different evidence requirements across public and private sectors and the lack of a clear reimbursement pathway for DMHTs may reduce investment in new technologies and threaten innovation.

Barriers to Expanding Access

Last year the Commonwealth Fund partnered with Leavitt Partners, an HMA Company, to conduct a landscape assessment of key statutory and regulatory authorities across the DMHT life cycle, including research and development, consumer access and Food and Drug Administration (FDA) regulatory approaches, reimbursement, utilization from the perspective of federal programs, as well as broader activities and authorities addressing data and compliance, which may affect the uptake and utilization of DMHT. Leavitt Partners engaged DMHT vendors, congressional staff, administration officials, advocates, and other cross-sector experts to better understand how federal statutory authorities and regulations are being utilized and where gaps or opportunities exist for new or improved legislative or regulatory actions. In November 2024, Leavitt Partners published a subsequent report[10] titled "Digital Mental Health Technologies: Gaps and Opportunities in Current U.S. Regulatory Authorities," identifying four key barriers to access to DMHT.

First, only a small portion of DMHT clearly falls under current regulations for consumer access, and the mechanisms for reviewing those that do are outdated. Tools classified as a "device" and "intended for use in the diagnosis of disease or other conditions, or in the cure, mitigation, treatment, or prevention of disease," are subject to FDA regulation. But most digital health apps are focused on "wellness

management," which means that the vast majority—estimated at more than 90,000 in 2020—are not subject to FDA oversight. Thus, there is little information to guide purchase decisions or address concerns that these tools' potential risks.

Second, the mechanisms for FDA review do not consider the rapid changes experienced in tools that utilize artificial intelligence (AI) to improve their usability and performance or to deliver and support care. FDA noted the challenges in a 2022 report, where they called for new regulatory authority to augment existing rules: "A flexible, risk-based approach to regulation could allow FDA to tailor regulatory requirements more efficiently for devices based on the latest science, the benefits and risks posed by devices, their real-world performance, and their contribution to promoting health equity."

Third, DMHT developers reported apparent lack of coordination across federal entities. When federal staff focus on the specific purview of their enabling regulation—as they should—stakeholders see a lack of coordination that requires costly efforts to meet needs of different agencies. It is clear federal stakeholders are working closely within their mandates, as evidenced by the new reimbursement codes for a targeted set of DMHTs included in the CMS 2025 Physician Fee Schedule. But the distinctions in authorizing legislation and regulations pose challenges for creating the evidence needed to address FDA and CMS requirements or expectations, particularly for smaller companies with limited resources. Further coordination and clarity will help drive government and private sector efficiency.

Fourth, data privacy and security arose as a key concern among a number of stakeholders, especially given the highly sensitive nature of data potentially incorporated in DMHTs, recent large data breaches affecting health care entities, and the growing use of AI. Health care privacy and security rules do not apply when tools are not controlled by a health care entity, and it can be difficult for consumers, clinicians, and even vendors to know when these rules apply. The Federal Trade Commission's role in protecting privacy and personal information is also relevant. AI tools, especially generative AI, contribute to worries about how companies may be taking advantage of personal information.

Recommendations for Executive Action

While the development of a comprehensive and more efficient federal regulatory framework for DMHT requires more public-private sector partnership and potentially new authorities from Congress, we have identified four steps the Administration could take to leverage existing authorities:

1. **Issue an Executive Order to Expand Agency Collaboration and Maximize Use of Existing Authorities.** In addition to FDA, there are other federal agencies with broad, flexible authorities that generally support DMHTs across the product life cycle, including basic and advanced research, development of evidence on the quality and function of the product, data and privacy protection, and reimbursement. Each of these agencies have different legal authorities, objectives, and missions that may necessitate different data and approaches, which may make it challenging for vendors to understand how to engage with each agency. By issuing an Executive Order, the President could direct HHS and other relevant agencies to review their existing authorities and practices and leverage these authorities to improve agency collaboration and further support the use of DMHT. For example, the Public Health Service Act (PHSA) provides broad authority for HHS to support, conduct, and coordinate research and development for a range of health issues, including DMHT and evidence development. Other statutes provide authorities related to coordinating cross-government federal research policy objectives, such as the Office of Science and Technology Policy (OSTP) and advancing early-stage research of novel technologies.

2. **Prioritize Policies to Expand Reimbursement of DMHTs.** Stakeholders view reimbursement or payment from public and private payors as necessary to further develop the DMHT market. In addition to payment for the product, some vendors and providers noted that reimbursement for clinician time spent in educating patients and using the product is an important factor. CMS recently included, in the final rule for the Medicare Physician Fee Schedule (PFS) for Calendar Year 2025, three billing codes for professional services related to DMHT. Though limited to DMHT with FDA clearance for use in treating a subset of mental health conditions, these billing codes could have broader impacts across the industry. Stakeholders generally viewed the initial proposal as progress, but some noted the scope of prescription DMHT for treatment as overly narrow and expressed concern that a new code will not guarantee uptake. The Center for Medicaid and Children's Health Insurance Program (CHIP) Services issued an informational brief in June 2024 that outlined opportunities for state Medicaid agencies to receive enhanced federal matching for health information technology expenditures related to increasing access to MH/SUD services, including smartphone and web applications, screening technology, and peer support web platforms. We recommend that HHS continue these policies and seek additional opportunities through the President's Budget and other vehicles to increase resources for reimbursement by federal health programs and address regulatory burdens related to the reimbursement process.

3. **Request Information on Evidence Requirements and Gaps.** Different evidence requirements across FDA, CMS, states, and other actors in the health care industry can present a barrier to DMHT advancement for startups and smaller companies. Insurers, employers, and other purchasers seek strong evidence on real-world effectiveness and impact on health outcomes and costs to decide whether to pay for a product. Though FDA assesses safety and efficacy of a DMHT, health care systems are interested in the broader impacts of DMHT. Health systems and payers will also consider whether a technology is better than the standard of care as they approach payment for these tools. Some participants noted that FDA's approval requirements and process is geared toward the traditional medical product industry versus DMHT developers. The lack of agreement about required evidence and trial methods for DMHT is also challenging, especially when DMHTs which incorporate AI tools are frequently updated to support ease of use. Patient groups and vendors need additional information on how agencies, such as the Health Resources and Services Administration and Substance Abuse and Mental Health Services Administration, as well as state Medicaid programs, approach the use of such technologies in programs that they support, the evidence required for use, and how these policies align with one another across the U.S. government. We recommend that HHS make a Request for Information from stakeholders, including patients, vendors, providers, and payers, to identify areas where additional clarity is needed to understand differing evidence requirements and to identify gaps in outcomes and quality data that stakeholders can be helpful in developing.

4. **Ensuring Privacy and Protection of Patient Data** The federal government should provide more clarity and awareness regarding data privacy and security requirements for digital tools, particularly those that include data related to mental health and substance use and including the use of patient and provider data in tools that use artificial intelligence (AI) methods. In addition, vendors and others should take steps to improve consumer and provider awareness of data privacy and security requirements. Depending on how the DMHT product is used, it may fall under the HIPAA privacy and security rule and/or FTC requirements. The health information ransomware data breach in early 2024 heightened attention to data security issues, after which the FTC finalized the Health Breach

Notification Rule (16 CFR Part 318) first proposed in June 2023. In the wake of this incident, increasing consumer awareness about the privacy protections to which specific products are subject will help bolster confidence and trust in the technology. The administration and vendors should consider whether sensitive information may be collected and/or shared as part of a DMHT product that falls outside these means of protection. The administration could partner with private sector organizations to increase consumer awareness of data privacy and security requirements, particularly as it relates to mental health and substance use.

Ensuring DMHTs are safe, effective, and secure is an important opportunity for the federal government to both advance the effective and responsible use of technology and to address the continuing mental health crisis that threatens our country's productivity and the well-being of our citizens. We look forward to working with you, other policymakers, and private sector stakeholders to support a tech-enabled holistic approach to health and wellness, which prioritizes patient outcomes and value.

Please reach out to us at rn@cmwf.org and rw@cmwf.org with any questions.

Sincerely,

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